Patient ID	: QLD25/6510	Sample ID	: 25051105
Name	**: MR MAHESH**	Registration Date	: 24/05/2025 11:45 AM
Age/Gender	: 36 Yrs/Male	Report Date	: 24/05/2025 01:07 PM
Ref. Dr.	: SELF	QLD25/6510	
Center	: Q LABS DIAGNOSTICS		

HAEMATOLOGY REPORT

Test Description	Result	Unit	Biological Reference Range
COMPLETE BLOOD PICTURE			
Hemoglobin	13.7	gm/dL	13.0 - 17.0
RBC Count	4.79	mil/cu.mm	4.0 - 5.5
Hematocrit HCT	40.8	%	40 - 54
Red Cell Indices			
Mean Corp Volume MCV	85.2	fL	83 - 101
Mean Corp Hb MCH	28.6	pg	27 - 32
Mean Corp Hb Conc MCHC	33.6	gm/dL	31.5 - 34.5
RDW-CV	13.8	%	11.6 - 14.0
Platelet Count	**0.56**	lac/cmm	1.5 - 4.5
Total Count and DIFFERENTIAL LEUCOCYTE COUNT			
Total WBC Count	**2900**	cell/cu.mm	4000 - 10000
Neutrophils	70	%	40 - 75
Lymphocytes	27	%	20 - 40
Eosinophils	01	%	01 - 06
Monocytes	02	%	02 - 10
Basophils	00	%	00 - 02
Microscopic Blood Picture			
RBC Morphology	Normocytic Normochromic		
WBC Morphology	**Leucopenia**		
Platelet on smear	**Thrombocytopenia**		

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BIOCHEMISTRY REPORT

Test Description	Result	Unit	Biological Reference Range
KIDNEY FUNCTION TEST(KFT)			
Serum Creatinine *Method: Modified Jaffe with no deproteinzation*	0.76	mg/dL	0.70 - 1.40
Urea *Method: Urease UV GLDH*	19.87	mg/dl	10.0 - 45.0
Blood Urea Nitrogen-BUN *Method: Calculated*	7.9	mg/dl	7 - 20
Uric Acid *Method: Uricase Peroxidase*	**2.84**	mg/dl	3.6 - 7.7
Sodium *Method: ISE*	**127**	mEq/L	135 - 146
Potassium *Method: ISE*	**2.3**	mEq/L	3.5 - 5.5
Chloride	**119**	mEq/L	96.0 - 106

Interpretation :

Kidney function tests help to screen the individual for renal disease and to determine the extent or progression of renal disease. These tests also aid in determining drug dosage for drugs excreted through the kidneys. The clinical syndrome resulting from decreased renal function and azotemia is called uremia Renal azotemia: glomerular nephritis and chronic pyelonephritis. Prerenal azotemia: severe dehydration, hemorrhagic shock, and excessive protein intake. Post renal azotemia: urethral stones or tumors and prostatic obstructions Measurement of urea in dialysis fluids is widely used in assessing the adequacy of renal replacement therapy.

In these prerenal situations, the plasma creatinine concentration may be normal.In obstructive post renal conditions, both plasma creatinine and urea concentrations will be increased, although there is often a greater increase in plasma urea than creatinine because of the increased back diffusion.These considerations give rise to the principal clinical utility of plasma urea, which lies in its measurement in conjunction with that of plasma creatinine and subsequent calculation of the urea nitrogen/creatinine ratio. This ratio has been used as a crude discriminator between prerenal and postrenal azotemia. Significantly lower ratios usually denote (1) acute tubular necrosis, (2) low protein intake, (3) starvation, or (4) severe liver disease (decreased urea synthesis). So even though blood urea is not an excellent marker of renal dysfunction as it rises quite late in the dysfunction and its rise is also not exclusive to kidney dysfunction, but for practical purposes serum urea level is still one of the most ordered test and forms an important part of the kidney function test.

Concentrations in excess of 6.0 mg/dL at 32 weeks gestation have been noted to be associated with a high perinatal mortality rate.

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LIPID PROFILE

Cholesterol-Total *Method: Spectrophotometry*	163.8	mg/dL	< 200 - Desirable 200 - 239 -Boderline High > 240 - High
Triglycerides level *Method: Serum, Enzymatic, endpoint*	87.2	mg/dL	< 150 - Normal 150 - 199 -Boderline High 200 - 499 -High >500 Very -High
HDL Cholesterol *Method: Serum, Direct measure-PEG*	32.8	mg/dL	< 40 - Low > 40 - Normal
LDL Cholesterol *Method: Enzymatic selective protection*	117.44	mg/dL	< 100 - Optimal 100 - 129 - Near/Above Optimal 130 - 159 - Borderline high 160 - 189 - High > 190 - Very High
VLDL Cholesterol *Method: Serum, Enzymatic*	17.44	mg/dL	6 - 38
CHOL/HDL RATIO *Method: Serum, Enzymatic*	4.99		0.27 - 5.0
LDL/HDL RATIO *Method: Serum, Enzymatic*	**3.58**		2.5 - 3.5

NOTE

8-10 hours fasting sample is required

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LIVER FUNCTION TEST (LFT)

TOTAL BILIRUBIN *Method: Serum, Jendrassik Grof*	**4.21**	mg/dl	0.2 - 1.2
DIRECT BILIRUBIN *Method: Serum, Diazotization*	**1.21**	mg/dL	0.0 - 0.3
INDIRECT BILIRUBIN *Method: Serum, Calculated*	**3.00**	mg/dl	0.0 - 1.0
SGPT (ALT) *Method: Serum, UV with P5P, IFCC 37 degree*	30.98	U/L	0 - 49
SGOT (AST) *Method: Serum, UV with P5P, IFCC 37 degree*	**98.26**	U/L	0 - 46
ALKALINE PHOSPHATASE *Method: DGKC*	88.74	U/L	40 - 125
TOTAL PROTEIN *Method: Serum, Biuret, reagent blank end point*	7.02	g/dl	6.0 - 8.3
SERUM ALBUMIN *Method: Serum, Bromocresol green*	4.15	g/dl	3.4 - 5.4
SERUM GLOBULIN *Method: Serum, Calculated*	2.87	g/dl	1.8 - 3.6
A/G RATIO *Method: Serum, Calculated*	1.45		1.2 - 2.2

NOTE :

In known cases of Chronic Liver disease due to Viral Hepatitis B & C, Alcoholic liver disease or Non alcoholic fatty liver disease, Enhanced liver fibrosis (ELF) test may be used to evaluate liver fibrosis.

**** End of the report. ****
Please correlate clinically.

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